



PRACTICE AND TEACHING

Logotherapeutically Oriented Counseling for Burnout and Crises of Meaning

Main Features, Procedure, and Limitations – With a Constructed Teaching Vignette

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English translation. The German original is the version of record.

ABSTRACT

In the literature, burnout is described as a syndrome of emotional exhaustion, depersonalization or cynicism, and reduced personal accomplishment, and it is classified in ICD-11 as an occupational phenomenon rather than a medical condition. Beyond organizational working conditions, an existential dimension was pointed out early on: burnout can be understood as a consequence of disappointed expectations of meaning in one's work. This contribution on practice and teaching outlines principles of logotherapeutically informed counseling for burnout and crises of meaning. Following a conceptual clarification, central logotherapeutic concepts are made accessible for counseling, including the existential vacuum, the distinction between meaning and success, the three categories of values, and dereflexion. The methodological approach is presented in four phases: clarification and differential assessment, relief and stabilization, exploration of meaning and values in Socratic dialogue, and implementation and transfer. An explicitly constructed teaching vignette, which does not represent any real counseling case, serves as an illustration. Particular attention is paid to the boundary between counseling and psychotherapy, especially the need to recognize depressive disorders and other conditions requiring treatment and to refer clients where appropriate, as well as to the risk of individualizing structural stressors.

Keywords: burnout · crisis of meaning · logotherapeutic counseling · Socratic dialogue · counseling psychology

ZUSAMMENFASSUNG

Burnout wird in der Fachliteratur als Syndrom aus emotionaler Erschöpfung, Depersonalisation bzw. Zynismus und reduziertem Leistungserleben beschrieben und in der ICD-11 als berufsbezogenes Phänomen, nicht als medizinische Erkrankung, klassifiziert. Neben arbeitsorganisatorischen Bedingungen wurde früh auf eine existenzielle Dimension hingewiesen: Burnout kann als Folge enttäuschter Sinnerwartungen an die eigene Arbeit verstanden werden. Der vorliegende Beitrag aus Praxis und Lehre entwickelt Grundzüge einer logotherapeutisch orientierten Beratung bei Burnout und Sinnkrisen. Nach einer begrifflichen Einordnung werden zentrale logotherapeutische Konzepte für die Beratung erschlossen, darunter das existenzielle Vakuum, die Unterscheidung zwischen Sinn und Erfolg, die drei Wertkategorien sowie die Dereflexion. Das methodische Vorgehen wird in vier Phasen dargestellt: Klärung und Abgrenzung, Entlastung und Stabilisierung, Sinn- und Werteexploration im sokratischen Dialog sowie Umsetzung und Transfer. Zur Veranschaulichung dient eine ausdrücklich konstruierte Lehrvignette, die keinen realen Beratungsfall wiedergibt. Besondere Aufmerksamkeit gilt der Abgrenzung von Beratung und Psychotherapie, insbesondere der Notwendigkeit, depressive Störungen und andere behandlungsbedürftige Zustände zu erkennen und gegebenenfalls weiterzuverweisen, sowie der Gefahr, strukturelle Belastungen zu individualisieren.

Schlüsselwörter: Burnout · Sinnkrise · Logotherapeutische Beratung · Sokratischer Dialog · Beratungspsychologie

1 Introduction and Conceptual Classification

Since the 1970s, the concept of burnout has become widespread in professional discussion and among the general public. With the Maslach Burnout Inventory, Maslach and Jackson (1981) presented a measurement instrument that captures burnout through the dimensions of emotional exhaustion,

depersonalization, and reduced personal accomplishment. In a later review, Maslach et al. (2001) emphasized the importance of the fit between person and work environment, for instance with regard to workload, control, reward, community, fairness, and values. In ICD-11, the World Health Organization lists burnout as an occupational

phenomenon in the chapter on factors influencing health status, and explicitly not as a medical condition (World Health Organization, 2019).

The distinction between burnout and depressive disorders is controversial in research and of great importance for practice (Schaufeli & Enzmann, 1998). The present contribution therefore explicitly does not understand counseling as the treatment of mental disorders. It is addressed to professionals in counseling contexts and describes how logotherapeutic concepts can be used in work-related exhaustion and crises of meaning – in awareness of the limits of this framework.

2 The Existential Dimension of Burnout

Pines (1993) interpreted burnout from an existential perspective: people who seek an essential source of meaning in their work are said to be particularly at risk when they experience that their efforts do not have the significance they had hoped for. From this point of view, burnout appears less as the result of mere overload than as the result of disappointed expectations of meaning. This interpretation is related to Frankl's concept of the existential vacuum, which describes a feeling of inner emptiness and meaninglessness (Frankl, 1969).

Also significant from a logotherapeutic standpoint is Frankl's distinction between meaning and success. Anyone who measures the value of their own work exclusively by measurable results, recognition, or efficiency makes their experience of meaning dependent on conditions that frequently lie outside their own control. Existential analysis has differentiated this further and has pointed to the importance of inner consent to one's own activity (Längle, 2013). According to this understanding, activity that is successful but carried out without inner consent can be exhausting in the long run.

Nevertheless, caution is called for: an existential interpretation must not lead to individualizing structural causes such as work intensification, a lack of resources, or unfair organizational conditions. Meaning-centered counseling does not replace a change in stressful working conditions; at most, it can support the person in recognizing and using their scope for action.

3 Logotherapeutic Concepts for Counseling

Several logotherapeutic concepts are particularly helpful for counseling in burnout and crises of meaning. Frankl's three categories of values (Frankl, 1946) open up a differentiated view of sources of

meaning: creative values are realized in work itself but are not restricted to it; experiential values in relationships, nature, or culture can form a counter-movement to a way of life one-sidedly geared toward performance; attitudinal values concern the stance toward burdens that cannot be changed immediately. Schnell (2020) has also pointed out that a greater diversity of sources of meaning may be associated with a more stable experience of meaning.

A further concept is dereflection. It aims at dissolving excessive, worried self-observation by directing attention to meaningful tasks and relationships beyond one's own state of mind. Lukas (1998) described in detail the methodical application of dereflection and other logotherapeutic procedures for counseling and therapy. In exhaustion, however, dereflection must be used cautiously: it must not be misunderstood as an invitation to ignore the body's warning signals.

It is also helpful to ask whether meaning is derived exclusively from work or whether other areas of life contribute to the experience of meaning. People with very high occupational identification often derive a large part of their experience of meaning from work. If this source falls into crisis, alternatives are lacking. Counseling can help to broaden the spectrum of possible sources of meaning again without devaluing the importance of work.

4 Methodical Procedure in Four Phases

The first phase serves clarification and delineation. Here, concerns, stress factors, and resources are assessed. At the same time, it must be carefully examined whether there are indications of a depressive disorder, a substance use problem, suicidality, or physical illnesses that require medical or psychotherapeutic evaluation. In these cases, counseling cannot replace treatment and should work toward an appropriate referral.

The second phase aims at relief and stabilization. Recovery, sleep, social support, and, where appropriate, conversations with supervisors or workplace contact persons are in the foreground. In this phase, questions of meaning are acknowledged rather than explored in depth, since an acutely exhausted person often lacks the strength for a fundamental reorientation.

In the third phase, meaning and values are explored, preferably in Socratic dialogue. Understood logotherapeutically, this does not aim at ascribing meaning to the person seeking advice but at enabling them, through questions, to discover their own

experiences of value and their own possibilities. Here there are points of contact with guided discovery in cognitive behavioral therapy, which likewise works with systematic questioning (Kazantzis et al., 2014; Overholser, 1993). The difference lies in the subject matter: whereas Socratic dialogue in cognitive behavioral therapy serves primarily to examine beliefs, in logotherapy it is directed toward values, responsibility, and possibilities of meaning.

The fourth phase serves implementation and transfer. Together, concrete steps are developed that carry what has been recognized over into everyday life: a changed weighting of areas of life, clarification of professional roles, cultivation of neglected experiential values, or even the decision for a professional reorientation.

It is important that the steps remain realistic and do not intensify the exhaustion through new performance demands. Logotherapeutically oriented counseling asks not only what a person should do but also what they may let go of and what stance they wish to take toward what cannot be changed for the time being.

5 Constructed Teaching Vignette

Note: The following vignette is a presentation constructed for teaching purposes. It does not reproduce a real counseling case; any resemblance to actual persons is unintended.

An experienced nurse on an internal medicine ward seeks out a counseling center because she has been feeling exhausted, irritable, and inwardly empty for months. She reports that she used to experience her profession as a vocation but now has the impression that she is “just functioning.” In the first phase, the counselor clarifies whether there are indications of a depressive disorder and recommends an evaluation by a general practitioner, which the client also follows up on. In the stabilization phase, possibilities for relief are discussed, including a conversation with the ward manager about staff scheduling.

In the exploration phase, the counselor asks in Socratic style in which moments at work the client most recently had the impression that what she was doing mattered. The client remembers a conversation

with a seriously ill patient for which she had taken a few minutes despite time pressure. In the further dialogue, it becomes clear that what is meaningful to her is less the number of tasks completed than the quality of the encounter, and that it is precisely this that falls short under the current conditions. In the transfer phase, she develops concrete steps to shape such encounters more consciously and at the same time decides to raise the structural burdens within the team.

For teaching, the vignette illustrates three points: the necessity of careful clarification at the outset, the combination of individual exploration of meaning with naming structural burdens, and the function of Socratic dialogue as a method of discovery rather than instruction.

6 Limitations and Conclusions

Logotherapeutically oriented counseling can offer people with work-related exhaustion and crises of meaning a differentiated framework for understanding their situation and discovering possibilities for action. Its efficacy in the context of burnout, however, has not yet been demonstrated in controlled studies; the main features presented here are therefore to be understood as a theoretically grounded and practice-oriented guide, not as an evidence-based treatment program. For continuing education, the task arises of strengthening, in addition to conveying logotherapeutic concepts, in particular the competencies for distinguishing conditions requiring treatment and for reflecting on organizational framework conditions.

For teaching, the procedure described is particularly suitable for practicing a questioning rather than an instructive style of conversation. In role-plays, students can try out how questions about experiences of value can be formulated without suggesting answers, and how indications of conditions requiring treatment can be recognized in conversation. Supervision and self-reflection are indispensable here, because work on questions of meaning touches on the counselors' own values, and there is a risk of transferring these unnoticed.

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